

Naming the bias

The senior names one out loud in the second pass; the scribe writes it in Bias watch. Naming it is the intervention — you cannot argue with a bias you have not said out loud.

BIASES THAT BUILD THE WRONG LIST

Anchoring

Locking onto the first feature you hear and not adjusting as new data arrives. The hip positioning fixes the list before the CBC is back.

→ *What if the first thing I heard were wrong?*

Availability

Judging likelihood by what comes to mind easily — usually the case you saw last month, not the one that is actually common.

→ *Am I picking this because it is likely, or because it is recent?*

Framing

The way the story is told shapes the list. “Frequent flyer with asthma” and “child with respiratory distress” produce different differentials.

→ *How would I list this if I had been told nothing about the patient?*

Diagnosis momentum

A label attached at triage, in the ED, or by the transferring hospital hardens into fact as it gets passed along.

→ *Who first said that word, and on what evidence?*

Triage cueing

Where the patient landed drives the thinking. A child sent to the floor is reasoned about differently from the same child sent to the PICU.

→ *Would my list change if they were one unit over?*

Base rate neglect

Ignoring how common things actually are — chasing the zebra, or dismissing the common thing because it feels boring.

→ *How common is this, really, in a child this age?*

AROUND THE ROOM

Visceral bias

Feelings about the patient or family — warm or otherwise — leaking into the assessment.

→ *Would my plan change if I felt differently about this family?*

BIASES THAT STOP YOU PRUNING

Premature closure

Accepting a diagnosis before it is verified. The single commonest reasoning error in medicine: when the diagnosis is made, the thinking stops.

→ *What else could this be, and what would prove it?*

Confirmation bias

Hunting for data that supports the leading diagnosis instead of data that would refute it.

→ *What finding would argue against my leading diagnosis?*

Search satisficing

Calling off the search once something is found. The second fracture, the co-infection, the second drug.

→ *If I found one, could there be another?*

Representativeness restraint

Dismissing a diagnosis because the presentation is not the textbook one — incomplete Kawasaki, the afebrile septic neonate.

→ *Is this an atypical version of something common?*

Sutton's slip

Going where the money is. The obvious answer arrives first and the alternatives never get considered.

→ *What have I not considered because the obvious answer got there first?*

Overconfidence

Acting on incomplete information because it feels sufficient. Often shows up as a plan with no stated trigger.

→ *What do I still not know?*

Hindsight bias

Once the answer is known it looks like it should have been obvious. This one distorts the reveal, not the reasoning.

→ *Was that knowable at the time, with the data they actually had?*

THE ONE THAT MATTERS MOST

Premature closure accounts for more diagnostic error than the rest combined. If the senior can only name one, it should usually be this one — and the fix is a stated trigger: “what would change my mind is...”